

Is Veterinary Medicine Safe From AI?

The planning guide — the full scores by track, the six factors behind them, the honest trade-offs, and what to ask before you commit.

2.7

AI EXPOSURE · SMALL ANIMAL GENERAL PRACTICE
where 10 is most at risk

For anyone still choosing — read it straight through.



Before you read this

You're weighing a veterinary path — or a route into the profession — and you want to know what that choice actually looks like in an AI economy. That's the whole job of this guide.

It's written to you directly, whether you are the one choosing or a parent thinking it through alongside them. Where it says "you," a parent can read "them"; it's the same decision from either side.

What this is. A facts document, not a recommendation. It sets out what's known, where the evidence is contested, and where we might be wrong — and then how to choose well given all of it.

If you're the parent, a note on how to use it. Don't lead with the scores. Open a conversation with a risk number and one of two things happens: they shut down, or they get defensive about a choice they're already leaning toward. Read it yourself first, then share it as information you both react to, not a verdict you've reached. They're choosing, not you — the job is to widen what's been considered, not to narrow it.

And two things specific to veterinary medicine.

First, this is often a very long-held ambition. More than most careers here, veterinary medicine tends to be something decided on at nine and never revisited. That's worth respecting rather than dismantling — but it does mean the practical realities may never have been examined properly. This guide is for examining them.

Second, and handled carefully further down: the profession openly discusses mental health and wellbeing in a way most don't, because it has genuine occupational stressors that are unusual and worth understanding before committing. We've covered that further down without sensationalizing it, because it matters and because the profession itself has built real support around it.

The short answer

Yes on AI exposure, and this is one of the safest careers we score.

Small animal general practice scores **2.7** out of 10. Large animal and emergency work scores **2.5**. Veterinary nursing scores **3.0**.

The protection is unusually complete: physical work, licensed practice, deep owner trust, and a patient who cannot tell you what is wrong.

The reasons to think hard about veterinary medicine are entirely different ones — competitive entry, substantial training debt, and an emotional load the profession takes seriously. Choosing the version of this path that holds up — and going in with those realities in front of you — is most of the decision.

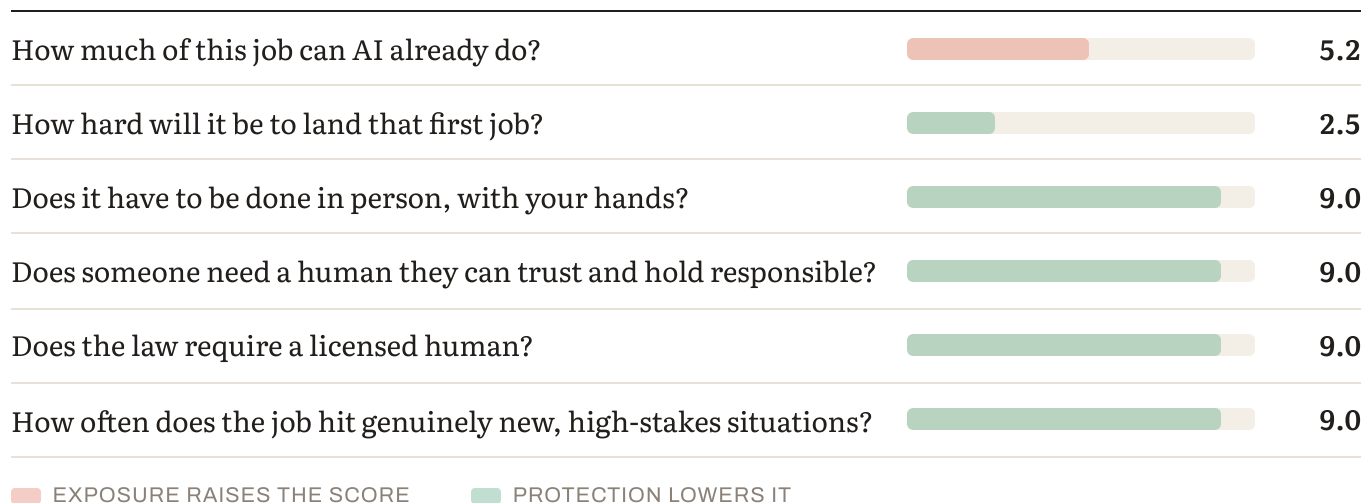
The scores

TRACK	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Emergency & critical care	2.0	2.2	2.4	+0.4	Very low
Large animal / mixed practice	2.0	2.2	2.5	+0.5	Very low
Veterinary specialist / surgery	2.0	2.2	2.5	+0.5	Very low
Small animal general practice	2.2	2.5	2.7	+0.5	Very low
Veterinary nurse / technician	2.5	2.7	3.0	+0.5	Low
Lab / diagnostic veterinary roles	4.6	5.0	5.4	+0.8	Moderate

Scores run 1–10, where 10 is most at risk. For context: the median profession in this edition sits around 5.5, surgery scores 2.1, physical therapy 2.5, bedside nursing 2.8, and entry-level software development 8.1.

Read the table this way, and note the last row. Veterinary work moved behind a microscope or a screen scores double the clinical tracks — the same pattern that appears in every profession we measure. Which is why, with veterinary, which setting you choose shapes the exposure more than the qualification does.

Why veterinary scores so low — the six factors



Here's how small animal general practice answers them.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

The administration, the imaging support, and very little else.

Clinical records and history notes. Imaging support and first-pass reads. Differential diagnosis suggestions. Practice admin, reminders and billing. Protocol and dosage lookup. Client communication drafting.

Veterinary practices are buried in administration, and the automation landing there is widely welcomed rather than feared.

What resists: physical examination of an animal that resists it, surgery, handling a frightened or aggressive patient, telling an owner their dog is dying, and judgment when the patient cannot describe anything.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Strong protection. Licensure requires supervised clinical training that cannot be automated or offshored, and veterinary school places are capped — which makes entry fiercely competitive but employment afterwards secure.

The bottleneck here is getting in, not getting hired after. Once qualified, employment is secure in a way almost nothing else in this index can claim — the hard part sits before the degree, not after it. The way through the competitive-entry wall isn't maximum volunteering hours: the AVMA has said explicitly that enormous quantities of unpaid experience aren't required for admission. It's meaningful animal exposure, a serious comparison of total program cost, and — once qualified — relief work and employer loan repayment treated as

deliberate strategies for the debt rather than afterthoughts. Those are concrete moves, and the career-hunting checklist further down lays them out.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

Almost entirely. Physical examination, procedures, surgery, restraint. The section on the patient who can't talk explains why this combines with something else to make veterinary unusually protected.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

Yes. Owners are making decisions about an animal they love, often under financial pressure, and a licensed veterinarian is professionally answerable for the outcome.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Comprehensively. Veterinary practice, prescribing and surgery are all legally reserved to licensed practitioners.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

We rate this 9.0, and veterinary earns it differently from every other career in this index. The section that follows explains why.

The patient can't talk — and why that matters more than it sounds

AI is strong on patterns and weak on genuine novelty. So this factor measures how often the work presents something that doesn't match anything in the training data, where being wrong is costly.

Most careers earn a high rating here through complexity — an engineer's one-off site, a litigator's unprecedented case, an ICU nurse's crashing patient.

Veterinary earns it through a structural absence: there is no history.

Human medicine begins with a patient describing symptoms — where it hurts, when it started, what makes it worse, what they've taken. That narrative is the single richest input in clinical reasoning, and it is text — which is precisely what language models handle best.

A veterinary patient supplies none of it. No complaint, no timeline, no symptom description. Instead there is an animal that may be hiding pain by instinct, an owner's second-hand and

often inaccurate account, and whatever the vet can determine by touching an animal that does not want to be touched.

Then multiply by species. A veterinarian in mixed practice may see a dog, a horse and a rabbit in one morning — three entirely different physiologies, drug tolerances and normal ranges.

That combination is unusually hostile to automation. Diagnostic support systems perform well when fed structured findings. Veterinary work is largely about *generating* those findings from a non-verbal, uncooperative, physically present patient — which no system can do.

The general lesson, and it transfers well beyond this profession: ask where a job's information comes from. Careers where the inputs arrive as text or structured data — analysts, paralegals, desk journalists — are exposed, because that is the form AI reads best. Careers where a human must generate the information from the physical world first are protected. Veterinary is the purest example of it in this index.

The financial picture — and it has been moving

This is the part that deserves the most attention, and the direction of travel matters as much as the level.

The AVMA's own data — the profession's annual Graduating Senior Survey — tells a two-part story.

It improved. The average debt-to-income ratio for new graduates entering full-time employment fell from 1.70 in 2021 to 1.44 in 2022, and stood at **1.4 for 2024 graduates**. That approaches the figure generally considered serviceable without serious financial stress.

Then it started climbing again. Average DVM debt across all new graduates — including those with none — rose to \$174,484 in 2025, up from \$153,972 in 2022. For those carrying debt, the average reached \$212,499, up from \$186,788.

The AVMA's own framing is blunt: a few years ago it looked as though the profession might be turning a corner, and more recent data show the picture becoming complicated again.

And the average conceals a wide spread. More than a quarter of new graduates in one recent cohort carried a debt-to-income ratio of 2.0 or higher — roughly double what professionals consider healthy, which is closer to 1.

Against that, median veterinary pay sits around \$125,510 according to BLS, with real geographic variation: Massachusetts, California and Hawaii average above \$155,000, while rural

states can run \$20,000–\$30,000 lower. Emergency and specialty practice pay more than general practice.

Two practical things worth knowing

Relief work changes the arithmetic. Relief veterinarians work per-diem shifts across multiple practices rather than as full-time employees. General practice relief rates run roughly \$55–\$125 an hour, with experienced practitioners typically \$70–\$95 and day rates of \$600–\$1,000. A relief vet working four days a week can earn \$120,000–\$160,000. It's a genuine strategy for accelerating debt repayment, and few applicants know it exists.

And the AVMA has explicitly told applicants they do not need enormous quantities of unpaid veterinary experience to be admitted. That guidance exists precisely because the expectation was pushing people into years of unpaid work before an already expensive degree. Worth knowing before spending two years volunteering on the assumption it's required.

Employer loan repayment assistance is also emerging — some large practice groups now contribute several hundred dollars a month toward veterinary student debt, which materially shortens a payoff timeline. It is a legitimate thing to ask about at interview.

What the trend shows

Small animal practice: 2.2 → 2.5 → 2.7. Up 0.5 in three years, and all of it administrative.

Lab and diagnostic roles: 4.6 → 5.0 → 5.4. Up 0.8.

The gap widened from 2.4 points to 2.7. Same pattern as everywhere: the further the work moves from the animal, the higher the exposure climbs.

How durable is the protection?

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Judgment under novelty	Among the highest we score	Most durable here	The absence of patient history is structural, not incidental.
Regulatory / licensing	Strong	Most durable	Practice, prescribing and surgery are legally reserved.
Human trust / accountability	Strong	Durable	Owners making hard decisions want a person answerable.
Physical / embodied	Very strong	Most fragile long-term	The protection physical AI targets — though animals are among the hardest subjects for robotics.

The honest read. Veterinary medicine has all four protections working together, which is rare. The physical one carries the usual long-horizon caveat, but restraining and examining an uncooperative animal is among the harder problems in robotics, not among the easier ones.

How the role will actually change

WAS THE JOB	IS BECOMING THE JOB
Writing up the consultation	Reviewing an ambient draft
Recalling drug doses across species	Instant lookup, with judgment about whether it applies
Reading the radiograph unaided	First-pass read arrives; the vet adjudicates
Practice admin consuming evenings	Substantially automated
Examining the animal	Unchanged
Telling an owner their animal is dying	Unchanged

The pattern is unusually clean here. In veterinary medicine the automatable layer and the clinical layer are cleanly separable, and what is automating is the paperwork — which is a leading complaint in the profession.

Human strengths that will matter most

1. **Physical examination of a non-verbal patient** — generating the clinical picture from scratch. The least automatable thing in this index. 2. **Handling animals** — restraint, reading fear and aggression, working safely with a patient that doesn't understand you're helping. 3. **Difficult conversations with owners** — euthanasia, cost, prognosis. Consistently named as the hardest and most human part of the work. 4. **Judgment across species** — knowing what normal looks like in three different physiologies in one morning. 5. **Accountability** — carrying professional responsibility for an animal's life.

And underneath all five, two traits the score can't grade: **curiosity and adaptability**. The specific tools — the ambient scribe, the imaging first-pass read, the diagnostic-support engine — will turn over many times across a veterinary career. The clinicians who compound are the ones who stay genuinely curious about the animal in front of them and re-tool fastest, rather than defending the one way of working they trained on. In a profession this well protected, that flexibility is less about survival than about staying good at the work as its tools change around you — and it's increasingly what employers hire for: the trajectory, not the current toolkit.

Other things to consider about this job

What's genuinely good about it

The work is described in vocational terms more consistently than almost any career we score. People don't drift into veterinary medicine; they choose it and mostly keep choosing it.

- **Extraordinary variety** — species, presentations, settings, and no two days alike
- **High autonomy**, and practice ownership as a realistic destination
- **Surgical and medical work combined** in a way human medicine separates
- **Immediate, visible results** in much of the work
- **Genuine relationships with clients** built over an animal's lifetime

And the financial picture is better than the debt figure alone suggests: BLS median around \$125,510, emergency and specialty practice paying above that, and relief work offering a route to \$120,000–\$160,000 with flexibility.

What's genuinely hard about it

Entry is fiercely competitive. Veterinary school places are capped and admission is demanding — in some markets harder than medical school.

The debt is substantial and rising again, covered in the financial picture above.

And the emotional load is real, unusual, and worth understanding properly before committing.

Three things make it distinctive:

- **Euthanasia is routine.** Few careers involve ending life regularly, and it does not become trivial.
- **The financial constraint on care.** A veterinarian frequently knows exactly what would help an animal and cannot provide it because the owner can't afford it. That situation has no real equivalent in human medicine, and practitioners consistently name it among the hardest parts of the job.
- **Client conflict,** sometimes severe, often at moments of grief or financial stress.

The profession discusses this openly, which is to its credit and unusual. The AVMA and equivalent bodies internationally run substantial wellbeing programs, peer support networks and confidential assistance services, and veterinary schools now build this into training rather than leaving graduates to discover it.

We'd rather a seventeen-year-old knew this going in than found out at twenty-six. It is not a reason to avoid the profession. It is a reason to enter it with open eyes, to take the support seriously, and to choose a practice culture carefully.

Who this work suits — and who it doesn't

Veterinary medicine tends to suit people who:

- **Are interested in animals and comfortable with people.** The most common misapprehension is that this is a career for people who prefer animals to humans. Much of the job is managing owners.
- **Are physically confident with animals** — including frightened, aggressive and large ones
- **Can hold competing pressures** — clinical, financial and emotional, in the same consultation
- **Are practical problem-solvers** rather than pure academics
- **Can carry difficult things** without being flattened by them

It's harder for people who:

- **Struggle with conflict**, particularly around money
- Find **grief** hard to be around repeatedly
- Want **predictable hours** — emergency and on-call work is common
- Chose it primarily because they **love animals**. That's the starting point, not the job.

What else is moving this market

Demand is stable and pet ownership is high, though the economics of veterinary care are under pressure from affordability at the client end.

Corporate consolidation of practices has changed the profession's structure considerably, with implications for autonomy, culture and ownership prospects that vary enormously by group.

And rural and large-animal practice faces persistent shortages, with government loan repayment programs available in high-need areas — worth investigating for anyone drawn that way.

The AI-native advantage — what to actually do about it

The situation: veterinary AI is arriving as administrative relief and diagnostic support rather than clinical substitution, which makes this one of the more straightforwardly positive pictures in the index. A good program should be building this fluency deliberately, not banning the tools or ignoring them.

WHAT AN AI-NATIVE VETERINARIAN LOOKS LIKE

- 1. Verify.** Knowing when an ambient consultation note or a first-pass image read is wrong. **A clinical skill, not a technical one — it depends on having examined the animal yourself.**
- 2. Use diagnostic support without deferring to it.** These tools are genuinely useful and genuinely confident when wrong.
- 3. Reclaim the administrative time deliberately.** Practice admin is the layer automating. Whether that time returns to patients or is absorbed by higher caseloads is partly a personal choice and partly a practice-culture question — worth asking about at interview.
- 4. Handle owner AI use.** Clients arrive having consulted symptom checkers and forums. **Being able to work with that without dismissing it is a genuinely new client-communication skill.**

5. Understand the limits. Diagnostic systems trained predominantly on one species or population behave unpredictably outside it, which matters more in veterinary than almost anywhere.

CONCRETE PREPARATION

Before applying: get real animal handling experience — but note the AVMA's guidance that enormous quantities of unpaid experience are not required for admission. Get meaningful exposure rather than maximum hours, and don't fund years of unpaid work you didn't need.

Choosing schools: compare total cost seriously. The debt spread between programs is substantial and it lasts decades.

During: get exposure across species and settings — small animal, large animal, emergency, shelter. The setting shapes the career more than the qualification does.

The routes in — and the versions that hold up

ROUTE	TYPICAL LENGTH	NOTES
DVM / veterinary degree	4 years post-degree, or 5–6 direct entry	The standard route. Entry is the hardest stage.
Veterinary nurse / technician	2–3 years	Hands-on animal care daily, far shorter and cheaper, scores 3.0. Genuinely under-considered.
Veterinary specialist training	+3–4 years post-qualification	Surgery, internal medicine, emergency, and others.
Large animal / rural practice	Same qualification	Persistent shortages, and government loan repayment programs exist in high-need areas.
Relief practice	Post-qualification	Higher day rates, flexible, and a real debt-repayment strategy.

For anyone drawn to animals rather than to medicine specifically, veterinary nursing offers hands-on animal care every day with a fraction of the training time and debt. Pay is lower and autonomy is less — but it is often the job people are actually picturing.

Where a veterinary degree can take you later

Broad: clinical practice across species and settings, specialization, practice ownership, research and academia, public health and epidemiology, government and regulatory work, industry (pharmaceutical and nutrition), conservation and wildlife, and shelter medicine.

The pattern in this index holds: the further from the animal, the higher the exposure. Lab and diagnostic roles score 5.4 against clinical practice at 2.5.

What to look for in a veterinary program

- 1. Total cost — and compare it seriously across schools.** The debt difference between programs lasts decades and dwarfs most other considerations.
 - 2. Clinical exposure: how much, how early, across how many species?**
 - 3. Support and wellbeing provision,** and how it's built into training rather than bolted on.
 - 4. Licensure exam pass rates and first-destination outcomes.**
 - 5. Attrition — and what the school says about why students leave.**
-

Questions to ask a veterinary program

On cost:

- What is the total cost of attendance for the full program, including living costs?
- What is the median debt at graduation for your graduates?

On clinical experience:

- How many weeks of clinical rotation, across how many species and settings?
- Is there large-animal and emergency exposure, or predominantly small animal?

On outcomes and support:

- What's your licensure exam first-time pass rate?
- What wellbeing and mental health support exists, and how is it integrated into training?

- What's your attrition rate, and what do students say about why they leave?

Red flags: vagueness about total cost or graduate debt; narrow species exposure; wellbeing support described only as a service rather than part of the curriculum.

The career-hunting checklist

Choosing well is step one. This is what turns a good choice into a good outcome once you're in — worth reading *now*, because the best time to pick a program is knowing exactly what you'll need to do with it. The first list is for the degree and training years; the second is for the job hunt that follows.

During the degree / training — build what can't be automated

- **Stay on the hands-on, animal-facing side of the work.** The clinical tracks — emergency and critical care (2.4), large animal and mixed practice (2.5), specialist and surgery (2.5), small animal general practice (2.7) — are the protected core. Lab and diagnostic roles (5.4) are the one lane that scores like an office job. Aim your exposure at the animal, not the screen.
- **Get exposure across species and settings.** Small animal, large animal, emergency, shelter. The setting shapes the career more than the qualification does, and breadth is the thing you can't easily add later.
- **Get meaningful animal experience, not maximum hours.** The AVMA has said enormous quantities of unpaid experience aren't required for admission. Get real exposure — and don't fund years of unpaid work you didn't need.
- **Compare total program cost seriously, and plan for the debt now.** The spread between programs lasts decades; average debt for graduates carrying it reached \$212,499. Learn what relief work and employer loan repayment can do before you graduate, not after.
- **Take the wellbeing support seriously from day one.** Euthanasia, cost-constrained care and client conflict are real and unusual; the profession has built genuine support around them. Use it as a habit, not a last resort.
- **Get fluent with veterinary AI as a clinician, not a shortcut.** Learn to direct the ambient scribe and the first-pass image read, and to catch them when they're confidently wrong — verification depends on having examined the animal yourself.
- **Feed curiosity and adaptability on purpose.** The tools will turn over repeatedly; the clinician who stays genuinely curious about the animal and re-tools fastest is the one who

compounds.

- **Keep the frame wide.** Properly vet the adjacent paths — veterinary nursing (3.0), large-animal and rural practice, specialty training — before ruling them out. The job people are actually picturing is sometimes a shorter, cheaper one.

During the job hunt — aim, don't just apply

- **Target clinical, hands-on practice on purpose.** Small animal, mixed, emergency, specialty — the animal-facing work is where the protection lives, and it's most of the profession.
- **Read the setting, not just the offer.** A role that's mostly screen-based diagnostic reading drifts toward the lab and diagnostic lane (5.4). If you take one, take it deliberately, with your clinical hours and licensure in view — not as the default.
- **Don't grab a role just because it's on offer in an exposed lane.** A role being available doesn't make it safe. A practice or lab building roles around behind-the-screen diagnostic work rather than hands-on clinical care may simply be behind the curve — and that's exactly the kind of work that scores 5.4 in this profession. Availability can be a lagging indicator, not a green light.
- **Investigate relief work and employer loan repayment as real debt strategies.** Four days a week of relief can earn \$120,000–\$160,000; some practice groups now contribute toward student debt. Both are legitimate to ask about at interview.
- **Choose practice culture as carefully as specialty.** Whether reclaimed admin time returns to patients or is absorbed by higher caseloads is a culture question — ask it directly.
- **Ask the employer where AI is heading in the role.** Whether they're building the job around ambient notes and diagnostic support, or waiting to be forced to, tells you if the seat grows or is quietly being hollowed out.

Go deeper — further reading

- **AVMA — Economic State of the Veterinary Profession and student debt research** — American Veterinary Medical Association. The profession's own annual data on debt, starting salaries and debt-to-income ratios. The primary source for the financial picture here, and the most reliable available.

- **[Veterinarians — Occupational Outlook Handbook](#)** — US Bureau of Labor Statistics. Median pay, projected growth and entry requirements. Read the **Veterinary Technologists and Technicians** page alongside it.
- **[VIN Student Debt Center](#)** — the Veterinary Information Network's resource on educational debt and repayment. Practical rather than promotional, and written by people inside the profession.

The counterargument — read this one:

The strongest case against our 2.7 isn't that AI will replace veterinarians. It's that a very low AI score may lead a family to under-weight the two risks that actually end veterinary careers: the debt and the emotional load.

On this reading, our framework tells a student that veterinary medicine is among the safest careers available — which is true — while saying nothing about a debt figure that has started climbing again, more than a quarter of recent graduates carrying debt-to-income ratios above 2.0, or the occupational stressors the profession itself treats as a serious wellbeing issue.

A career can be automation-proof and still be difficult to sustain.

Where we land: this is fair, and it's why the financial and emotional sections are longer here than the AI analysis. Our score measures automation exposure only. For veterinary medicine specifically, the AI answer is genuinely reassuring and the honest answer is more complicated — and conflating the two would be the most damaging thing this document could do.

Bottom line

On AI exposure, veterinary medicine is about as protected as work gets. All four of our protections operate together: physical examination, comprehensive licensure, deep owner trust, and a patient who cannot describe a single symptom. The absence of a spoken history is a structural protection nothing else in this index has.

The real considerations are financial and emotional, and both deserve serious weight. Debt averaging \$212,499 for graduates who carry it, rising again after several years of improvement, with more than a quarter of a recent cohort above a 2.0 debt-to-income ratio. And an emotional load — routine euthanasia, financial constraint on care, client conflict at moments of grief — that the profession has built substantial support around precisely because it is real.

So the useful advice is: compare total program cost seriously rather than reputation, because the difference lasts decades. Investigate relief work and employer loan repayment as genuine strategies rather than afterthoughts. Get meaningful animal experience rather than maximum hours, since the profession has said the latter isn't required. Choose practice culture as carefully as specialty. And take the wellbeing support seriously from the start rather than when it's needed.

And one thing worth saying plainly, if this has been the plan since childhood. **The instinct is a good one, and the career is genuinely protected in a way very few are. Go in with the numbers and the difficulties in front of you — not instead of the ambition, but alongside it.**

Questions worth sitting with

Whether you're the one choosing or a parent thinking it through together, these are the conversations that lead to a better decision. Read "you" as whoever the choice is about.

About the work itself

1. What made veterinary medicine appealing, and when? If it's a long-held ambition, has it ever been examined properly? 2. **Do you realize how much of the job is managing owners rather than animals?** This is the most common misapprehension about this career. 3. How do you feel about being physically confident with frightened or aggressive animals? 4. What does a Tuesday look like in the job you're imagining?

Reacting to the findings

5. Small animal practice scores 2.7 and lab work 5.4. Does the setting matter to you? 6. This guide argues the protection comes from the patient being unable to speak. Does that land — and can you apply the idea to another career? 7. Was anything here surprising?

The money conversation

8. **Have you looked at total cost across the schools you're considering?** The AVMA's own data shows average debt for graduates carrying it at \$212,499, and rising. 9. Do you know that more than a quarter of a recent graduating cohort had debt at twice their starting salary or more? 10. **Have you looked at relief work?** Four days a week can earn \$120,000–\$160,000, and it's a real debt strategy that few applicants know about. 11. Did you know the AVMA has said applicants don't need enormous unpaid experience hours to be admitted?

The harder conversation

12. **Euthanasia is routine.** Have you thought about that specifically, rather than in the abstract? 13. How would you handle knowing exactly what an animal needs and being unable to provide it because the owner can't pay? Practitioners name this among the hardest parts of the job. 14. The profession takes wellbeing seriously and has built real support around it. Would you use that support, or feel you should cope alone? Worth answering honestly now.

Widening the frame

15. **Have you looked seriously at veterinary nursing?** Hands-on animal care daily, two to three years, far less debt, scores 3.0. It is often the job people are actually picturing. 16. Have you considered large-animal or rural practice? Persistent shortages, and government loan repayment programs exist in high-need areas. 17. If veterinary medicine weren't available, what would you choose? The answer usually reveals what you're actually drawn to.

This is analysis and informed judgment about a fast-moving situation. For veterinary medicine specifically, we've flagged that the profession's most serious risks — the debt and the emotional demands — sit entirely outside what our six factors can measure, and that a low AI score should not be read as an easy career.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology. Scores measure exposure to what AI can already do — not how much any particular employer has deployed.

Technical scoring appendix — Veterinary Medicine

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

1. The formula

$$\begin{aligned}\text{Risk} = & 0.35 \times \text{Automatability} \\ & + 0.15 \times \text{EntryErosion} \\ & + 0.15 \times (10 - \text{Physical}) \\ & + 0.15 \times (10 - \text{Trust}) \\ & + 0.10 \times (10 - \text{Regulatory}) \\ & + 0.10 \times (10 - \text{Judgment})\end{aligned}$$

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-eight careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human?*

RATING	ANCHOR
0–2	No license exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent, where being wrong is costly?*

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

EMERGENCY & CRITICAL CARE — 2.4 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.4	4.8	5.3	1.85
Entry-path erosion	15%	1.2	1.5	1.8	0.27
Physical / embodied	15%	9.5	9.5	9.5	0.07
Human trust / accountability	15%	9.5	9.5	9.5	0.07
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	9.2	9.2	9.2	0.08
				Total	2.41 → 2.4

LARGE ANIMAL / MIXED PRACTICE — 2.5 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	4.7	5.4	1.89
Entry-path erosion	15%	1.2	1.5	1.8	0.27
Physical / embodied	15%	9.5	9.5	9.5	0.07
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	9.0	9.0	9.0	0.1
				Total	2.49 → 2.5

VETERINARY SPECIALIST / SURGERY — 2.5 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	4.7	5.4	1.89
Entry-path erosion	15%	1.2	1.5	1.8	0.27
Physical / embodied	15%	9.5	9.5	9.5	0.07
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	9.0	9.0	9.0	0.1
				Total	2.49 → 2.5

SMALL ANIMAL GENERAL PRACTICE — 2.7 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.6	5.3	5.8	2.03
Entry-path erosion	15%	1.5	1.8	2.0	0.3
Physical / embodied	15%	9.2	9.2	9.2	0.12
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	9.0	9.0	9.0	0.1
				Total	2.7 → 2.7

VETERINARY NURSE / TECHNICIAN — 3.0 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.4	4.9	5.6	1.96
Entry-path erosion	15%	1.8	2.0	2.2	0.33
Physical / embodied	15%	9.2	9.2	9.2	0.12
Human trust / accountability	15%	8.5	8.5	8.5	0.22
Regulatory / licensing	10%	8.5	8.5	8.5	0.15
Judgment under novelty	10%	8.0	8.0	8.0	0.2
				Total	2.98 → 3.0

LAB / DIAGNOSTIC VETERINARY ROLES — 5.4 (MODERATE)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.5	5.4	6.4	2.24
Entry-path erosion	15%	3.5	4.0	4.5	0.67
Physical / embodied	15%	4.0	4.0	4.0	0.9
Human trust / accountability	15%	5.0	5.0	5.0	0.75
Regulatory / licensing	10%	6.0	6.0	6.0	0.4
Judgment under novelty	10%	5.5	5.5	5.5	0.45
				Total	5.42 → 5.4

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Emergency & critical care

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	4.4	1.2	0.28	2.0
2025	4.8	1.5	0.28	2.2
Fall 2026	5.3	1.8	0.28	2.4

Movement decomposition: automatability contributed **+0.31** and entry-path erosion **+0.09**, for a total of **+0.4** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labor market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.